Falls Prevention Policy

Reference	NUR-FAL-02 v2
Owner	Falls Prevention Group
Approved by	Quality and Patient Safety Committee
Effective date	09/02/2026
Next review	09/02/2029
Applies to	All clinical areas of the Centre Hospitalier Saint-Aubin

1. Overview

This document describes the arrangements in force at the Centre Hospitalier Saint-Aubin for the topic named in its title. It does not contain clinical dosing, results or patient-specific information. Staff are asked to consult the relevant clinical protocols for patient care decisions.

2. Arrangements

Responsibilities are shared between the department concerned, the ward managers and the directorate. Operational details are held by the document owner.

3. Governance, audit and training

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

4. Revision history

Version	Date	Author	Summary of change
1.0	09/02/2021	Falls Prevention Group	First issue
2.0	09/02/2023	Falls Prevention Group	Triennial review; format harmonised
3.0	09/06/2025	Falls Prevention Group	Tables updated after clinical audit
current	09/02/2026	Falls Prevention Group	Review and re-issue

Annex A. Implementation notes

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

Annex B. Implementation notes

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Annex C. Implementation notes

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Annex D. Implementation notes

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

Annex E. Implementation notes

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

Annex F. Implementation notes

Staff who identify a deviation from this document, or a situation it does not cover, should discuss it with the senior clinician on duty and report it through the incident reporting system. Reporting is a learning tool and not a disciplinary process.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

Training on this document is part of the induction programme for clinical staff and is refreshed through the annual mandatory training cycle. Agency and locum staff receive a short briefing at the start of their first shift.

Equality and diversity: this document has been assessed for its effect on patients and staff with protected characteristics. Reasonable adjustments, interpreters and accessible formats are available through the patient experience team.

Data protection: any patient information handled under this document is processed in accordance with the data protection policy of the hospital. Access to records is limited to staff involved in the care of the patient.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.

The document owner reviews the content at least every three years or earlier when national guidance, a serious incident or an audit finding requires it. Interim changes are approved by the relevant committee chair and recorded in the revision history.

Where this document refers to a role (for example the ward pharmacist or the senior nurse), the post-holder on duty carries out the task or delegates it to a suitably trained colleague who remains accountable to them.

This document is controlled. Printed copies are uncontrolled: the current version is held on the hospital document management system and a printed copy must be checked against it before use.

Compliance is audited by the clinical audit department on a rolling sample of records. Results are reported to the quality and patient safety committee, which agrees actions with the ward managers and monitors their completion.